PEDIATRIC NURSING (THEORY) · LECTURE 12 · QUESTION BANK

Respiratory Distress Syndrome (RDS)

متلازمة الضائقة التنفسية عند حديثي الولادة — بنك الأسئلة

54	6	12
Past exam questions	NCLEX-style	Flashcards

المصادر اللي اتجمعت منها أسئلة المحاضرة دي:

ميدترم 2020 · ميدترم 2021 · ميدترم 2024 (ترم أول 2023-2024) · كويز 2 – 2022 · كويز 2 شعبة 3 (ملف Quiz 1 and 2) · كويز 2 – 2022 (ملف Quiz2 pediatric 2022) · أسئلة سابقة · تجميعة صور ميدترمات سابقة

جنب كل سؤال مصدره. لو السؤال اتكرر في كذا امتحان مكتوب كل مرة بمصدرها، وعليه علامة "تكرر". الإجابات كلها في آخر الملف.

اللَّهُمَّ لا سَهْلَ إِلَّا ما جَعَلْتَهُ سَهْلًا، وأنتَ تَجْعَلُ الحَزْنَ إذا شِئْتَ سَهْلًا — صلِّ على النبي ﷺ

Part A · Past Exam Questions

أسئلة الدفعات السابقة

1 (Matching) Grade I respiratory distress is manifested by: — ميدترم 2020

a. Rapid respiration and nasal flaring
b. Intercostal and subcostal retractions
c. Expiratory grunting
d. Cyanosis and altered conscious level
e. Wheezy chest and tachypnea

2 (Matching) Grade II respiratory distress is manifested by: — ميدترم 2020

a. Rapid respiration and nasal flaring
b. Intercostal and subcostal retractions
c. Expiratory grunting
d. Cyanosis and altered conscious level
e. Wheezy chest and tachypnea

3 (Matching) Grade III respiratory distress is manifested by: — ميدترم 2020

a. Rapid respiration and nasal flaring
b. Intercostal and subcostal retractions
c. Expiratory grunting
d. Cyanosis and altered conscious level
e. Wheezy chest and tachypnea

4 (Matching) Grade IV respiratory distress is manifested by: — ميدترم 2020

a. Rapid respiration and nasal flaring
b. Intercostal and subcostal retractions
c. Expiratory grunting
d. Cyanosis and altered conscious level
e. Wheezy chest and tachypnea

5 Which of the following findings would alert the nurse to notify the primary health care provider when assessing the neonate? تكرر في 2 مصادر — ميدترم 2021

a. Red reflex of the eyes
b. Equal bilateral air entry
c. Respiratory rate 40 br/min
d. Sternum retraction

6 Which of the following is a specific treatment of respiratory distress syndrome? تكرر في 2 مصادر — ميدترم 2021

a. Antibiotic medication
b. Mechanical ventilation
c. Exchange transfusion
d. Surfactant replacement

7 A neonate's oral cavity is filled with mucus and he became cyanosed. What is the first action of the nurse? تكرر في 2 مصادر — ميدترم 2024 (ترم أول 2023-2024)

a. Recording the incident
b. Insert nasogastric tube
c. Frequent suctioning
d. Administering O2 therapy

8 The nurse could teach the parents that the main function of surfactant is to: تكرر في 2 مصادر — ميدترم 2024 (ترم أول 2023-2024)

a. Decrease lung secretion
b. Prevent lung collapse
c. Produce lung collapse
d. Decrease lung inflation

9 Which assessment signs alert the nurse to the possibility of respiratory distress syndrome? تكرر في 2 مصادر — كويز 2 – 2022

a. Hypotension and bradycardia
b. Tachypnea and retractions
c. Acrocyanosis and grunting
d. The presence of a barrel chest

10 Which nursing diagnosis is priority for a preterm infant with respiratory distress syndrome? — كويز 2 – 2022

a. Fluid volume deficit
b. Risk for infection
c. Impaired gas exchange
d. Altered body temperature

11 While developing a plan of care for a newborn with RDS, which is considered the main nursing diagnosis? — كويز 2 شعبة 3 (ملف Quiz 1 and 2)

a. Body image disturbance
b. Altered body temperature
c. Impaired gas exchange
d. Altered bowel elimination

12 An early sign of neonatal respiratory distress syndrome is grunting. (True / False) تكرر في 2 مصادر — تجميعة صور ميدترمات سابقة

a. True
b. False

13 An early sign of neonatal respiratory distress syndrome is grunting. (True / False) تكرر في 2 مصادر — تجميعة صور ميدترمات سابقة

a. True
b. False

14 The nurse could teach the parents that the main function of surfactant is to تكرر في 2 مصادر أسئلة سابقة

a. Decrease lung secretion
b. Prevent the lung collapse
c. Produce lung collapse
d. Decrease lung inflation

15 A neonate's oral cavity is filled with mucus and he became cyanosed what is the first action of the nurse to do تكرر في 2 مصادر أسئلة سابقة

a. Recording the incident
b. Insert nasogastric tube
c. Frequently suctioning
d. Administering O2 therapy

16 Which assessment signs alert the nurse to the possibility of respiratory distress syndrome تكرر في 2 مصادر أسئلة سابقة

a. Hypotension and bradycardia
b. Tachypnea and retractions
c. Acrocyanosis and grunting
d. The presence of a barrel chest

17 The major main aim of surfactant therapy is as follow أسئلة سابقة

a. Increase air leaks of lung
b. Decreased oxygen requirement
c. Decrease surface tension of alveoli
d. Increase surface tension of alveoli

18 An early sign of neonatal respiratory distress syndrome is grunting. (True / False) تكرر في 2 مصادر أسئلة سابقة

a. True
b. False

19 In which weeks of gestation of the following for starting corticosteroid therapy for pregnant women to overcome fetal lung prematurity أسئلة سابقة

a. 20 - 30
b. 22 – 32
c. 24 – 34
d. 25 – 35

20 Which of the following nursing diagnosis is priority for preterm infant with respiratory distress syndrome? أسئلة سابقة

a. Fluid volume deficit
b. Risk for infection
c. Impaired gas exchange
d. Altered body temperature

21 Which of the following findings would alert the nurse to notify the primary health care provider when assessing the neonates? تكرر في 2 مصادر أسئلة سابقة

a. Red reflex of the eyes
b. Equal bilateral air entry
c. Respiratory rate 40br/min
d. Sternum retraction

22 The preterm infant is diagnosed with respiratory distress syndrome which is caused by alteration in the body's secretion of أسئلة سابقة

a. Progesterone
b. Adrenaline
c. Surfactant
d. Somatotropin

23 Which of the following is a specific treatment of respiratory distress syndrome

أسئلة سابقة

تكرر في 2 مصادر

a. Antibiotic medication
b. Mechanical ventilation
c. Exchange transfusion
d. Surfactant replacement

24 Hyaline membrane disease also called RDS

أسئلة سابقة

a. True
b. False

25 All of the following considered risk factors of RDS except

أسئلة سابقة

a. Cold stress
b. Maternal diabetes
c. Limited birth
d. Prematurity

26 Which of the following is considered a physiological abnormality of RDS?

أسئلة سابقة

a. Decreased alveolar surface tension
b. Decreased alveolar ventilation
c. Increased lung volume

27 Intercostal retraction is a sign to diagnose which grade of RDS

أسئلة سابقة

a. 3
b. 2
c. 1
d. 4

28 Antenatal corticosteroid therapy are indicated in pregnant women 24 – 34 week's gestation at high risk of preterm delivery within the next 7 days

أسئلة سابقة

a. True
b. False

29 Type of surfactant that is an extract of calf lung:

أسئلة سابقة

a. Curosurf
b. Survanta
c. Infasurf

30 What of the following consider nursing diagnosis of patient with RDS

أسئلة سابقة

a. Impaired gas exchange
b. Imbalanced nutrition less than body requirements
c. Ineffective family coping
d. All of the above

31 RDS is the most common cause of respiratory distress in premature infants, correlating with structural and functional immaturity of the heart. (True / False)

أسئلة سابقة

a. True
b. False

32 The more the gestational age and birth weight, the more the incidence of RDS. (True / False)

أسئلة سابقة

a. True
b. False

33 It's alse known as hyaline membrane disease:

أسئلة سابقة

a. Rheumatic fever
b. Acute kidney injury
c. Heart failure
d. Respiratory distress syndrome

34 **Pulmonary surfactant production begins at** أسئلة سابقة

a. 34 weeks **b.** 26 weeks
c. 24 weeks **d.** 32 weeks

35 **Infants born after 32 weeks have a much higher chance of survival than those born at 24 weeks.** أسئلة سابقة

a. True **b.** False

36 **Adequate amounts of pulmonary surfactant is produced to prevent atelectasis at about** أسئلة سابقة

a. 34 weeks **b.** 26 weeks
c. 24 weeks **d.** 32 weeks

37 **All of the following are considered predisposing and risk factor of RDS, Except:** أسئلة سابقة

a. Perinatal asphyxia **b.** Cold stress
c. Female gender **d.** Genetic disorders

38 **In RDS, Mortality rate increases with decreasing gestational age and in LBW newborns.** أسئلة سابقة

a. True **b.** False

39 **Mature levels of pulmonary surfactant are usually present after** أسئلة سابقة

a. 34 weeks **b.** 26 weeks
c. 35 weeks **d.** 24 weeks

40 **In RDS, the interference with gas exchange results in which of the following except:** أسئلة سابقة

a. Hypoxia (decreased O2 saturation) **b.** Increased PH
c. Increased PaCO2 **d.** Increased work of breathing

41 **A newborn infant with RDS may be presented with which of the following, except:** أسئلة سابقة

a. Tachypnea **b.** Nasal flaring
c. Hypertension **d.** Intercostal retraction

42 **In most cases of RDS, symptoms and signs reach a peak within 3 days** أسئلة سابقة

a. True **b.** False

43 **Which of the following electrolytes imbalance associated with RDS:** أسئلة سابقة

a. Respiratory acidosis **b.** Metabolic alkalosis
c. Respiratory alkalosis **d.** Metabolic acidosis

44 **A preterm infant presented with dyspnea, fatigue, nasal flaring, expiratory retraction, his respiratory rate is 85br/min, according to the S&S of the infant, the infant is in which grade of RD:** أسئلة سابقة

a. Grade I **b.** Grade II
c. Grade III **d.** Grade IIII

45 Neonatal respiratory distress syndrome (NRDS): أسئلة سابقة

a. Occurs in ~50% of premature births with a weight of <1500 g
b. Always follows premature births before 30-week gestation
c. Is commonly treated by giving the neonate corticosteroids
d. Occurs in 1 in 300 of all births.

46 Four 38-week-gestation gravidas have just delivered. Which of the babies should be monitored closely by the nurse for respiratory distress? أسئلة سابقة

a. The baby whose mother has diabetes mellitus.
b. The baby whose mother has lung cancer.
c. The baby whose mother has hypothyroidism.
d. The baby whose mother has asthma.

47 A newborn infant is presented by the following S&S, RR 70 br/min, mild retraction, no cyanosis, mild decrease in air entry, and audible grunting by stethoscope, according to Downe's score, the evaluation of this infant and his diagnosis is: أسئلة سابقة

a. 5 & respiratory distress
b. 3 & no respiratory distress
c. 4 & respiratory distress
d. 8 & impeding respiratory failure

48 For the prevention of RDS, the following measures are done, except: أسئلة سابقة

a. Administering tocolytics
b. Surfactant replacement therapy
c. Administering corticosteroids
d. Give beta blockers

49 A 1,000-gram neonate is being admitted to the neonatal intensive care unit. The surfactant Survanta (beractant) has just been prescribed to prevent respiratory distress syndrome. Which of the following actions should the nurse take while administering this medication? أسئلة سابقة

a. Flush the intravenous line with normal saline solution.
b. Assist the neonatologist during the intubation procedure.
c. Inject the medication deep into the vastus lateralis muscle.
d. Administer the reconstituted liquid via an oral syringe.

50 A 30-week-gestation neonate, 2 hours old, has received Survanta (beractant). Which of the following would indicate a positive response to the medication? أسئلة سابقة

a. Axillary temperature 98.0°F
b. Oxygen saturation 96%
c. Apical heart rate 154 bpm
d. Serum potassium 4.0 mEq/L

51 Administration of corticosteroids to women 48 hours before delivery of fetuses helps: أسئلة سابقة

a. Stimulate the production of surfactant
b. Accelerate fetal lung maturation
c. Reduce the incidence of RDS
d. All of the above

52 For newborn with RDS, aiming to keep pO2 between أسئلة سابقة

a. 40-70 mmHg
b. 50-80 mmHg
c. 55-90 mmHg
d. 45-75 mmHg

53 For an infant with RDS the proper position is: أسئلة سابقة

a. Side lying
b. Left lateral
c. Supine
d. Semi-Fowler's

54 The major main aim of surfactant therapy is: كويز 2 – 2022 (ملف Quiz2 pediatric 2022)

a. Increase air leaks of lung
b. Decreased oxygen requirement
c. Decrease surface tension of alveoli
d. Increase surface tension of alveoli

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

N1 The primary cause of RDS in preterm infants is: NCLEX

a. Infection
b. Surfactant deficiency
c. Meconium aspiration
d. Congenital heart disease

N2 Which is an EARLY sign of RDS? NCLEX

a. Grunting
b. Tachypnea
c. Cyanosis on room air
d. Apnea

N3 Select ALL risk factors for RDS. SELECT ALL

a. Prematurity
b. Maternal diabetes
c. Cold stress
d. Perinatal asphyxia
e. Female gender

N4 Antenatal corticosteroids are given to mothers at risk of preterm delivery between: NCLEX

a. 20–30 weeks
b. 24–34 weeks
c. 34–40 weeks
d. Any time

N5 Surfactant replacement is administered via: NCLEX

a. IV line
b. Endotracheal tube
c. Intramuscular injection
d. Oral syringe

N6 The expected ABG in an infant with RDS is: NCLEX

a. Respiratory alkalosis
b. Respiratory and metabolic acidosis
c. Metabolic alkalosis
d. Normal

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

RDS	Hyaline membrane disease; surfactant deficiency in preterm.	**Surfactant**	Decreases alveolar surface tension; prevents collapse.
Production	Begins ~24 wk; adequate ~32 wk; mature ~35 wk.	**Risk factors**	Prematurity, male, maternal DM, cold stress, asphyxia, C-section.

Grade I	Rapid respiration, nasal flaring.	**Grade II**	Intercostal/subcostal retractions.
Grade III	Expiratory grunting.	**Grade IV**	Cyanosis, altered consciousness.
Prevention	Antenatal corticosteroids 24–34 wk, tocolytics.	**Specific treatment**	Surfactant replacement via ET tube.
Support	CPAP, O2 keeping PaO2 50–80, NTE, minimal handling.	**Priority ND**	Impaired gas exchange.

Answer Key

الإجابات

1. a — Rapid respiration and nasal flaring
وصف الـ (سؤال توصيل في الامتحان الأصلي، اتحوّل لاختيار من متعدد. مكتوب من الملخص لأنه كان مقصوص في الصورة meningocele).

2. b — Intercostal and subcostal retractions
وصف الـ (سؤال توصيل في الامتحان الأصلي، اتحوّل لاختيار من متعدد. مكتوب من الملخص لأنه كان مقصوص في الصورة meningocele).

3. c — Expiratory grunting
وصف الـ (سؤال توصيل في الامتحان الأصلي، اتحوّل لاختيار من متعدد. مكتوب من الملخص لأنه كان مقصوص في الصورة meningocele).

4. d — Cyanosis and altered conscious level
وصف الـ (سؤال توصيل في الامتحان الأصلي، اتحوّل لاختيار من متعدد. مكتوب من الملخص لأنه كان مقصوص في الصورة meningocele).

5. d — Sternum retraction

6. d — Surfactant replacement

7. c — Frequent suctioning

8. b — Prevent lung collapse

9. b — Tachypnea and retractions

10. c — Impaired gas exchange

11. c — Impaired gas exchange

12. b — False
الـ grunting علامة متأخرة (Grade III) — المبكرة tachypnea.

13. b — False
الـ grunting علامة متأخرة (Grade III) — المبكرة tachypnea.

14. b — Prevent the lung collapse

15. c — Frequently suctioning

16. b — Tachypnea and retractions

17. c — Decrease surface tension of alveoli

18. b — False
الـ grunting علامة متأخرة (Grade III) — المبكرة tachypnea.

19. c — 24 – 34

20. c — Impaired gas exchange

21. d — Sternum retraction

22. c — Surfactant

23. d — Surfactant replacement

24. a — True

25. c — Limited birth

26. b — Decreased alveolar ventilation

27. b — 2

28. a — True

29. c — Infasurf
Infasurf = calfactant (من رئة العجل)؛ Survanta من رئة البقر، Curosurf من رئة الخنزير.

30. d — All of the above

31. b — False

32. b — False

33. d — Respiratory distress syndrome

34. c — 24 weeks

35. a — True

36. d — 32 weeks

37. c — Female gender

38. a — True

39. c — 35 weeks

40. b — Increased PH

41. c — Hypertension

42. a — True

43. d — Metabolic acidosis

44. b — Grade II

45. a — Occurs in ~50% of premature births with a weight of <1500 g

46. a — The baby whose mother has diabetes mellitus.

47. c — 4 & respiratory distress

48. d — Give beta blockers

49. b — Assist the neonatologist during the intubation procedure.

50. b — Oxygen saturation 96%

51. d — All of the above

52. b — 50-80 mmHg

53. d — Semi-Fowler's

54. c — Decrease surface tension of alveoli

N1. b — Surfactant deficiency
Lack of surfactant increases alveolar surface tension, causing collapse (atelectasis).

N2. b — Tachypnea
Tachypnea and nasal flaring appear early; grunting and cyanosis are later grades.

N3. a, b, c, d — Prematurity; Maternal diabetes; Cold stress; Perinatal asphyxia
Male gender is the risk factor.

N4. b — 24–34 weeks
Steroids accelerate fetal lung maturity and surfactant production.

N5. b — Endotracheal tube
The nurse assists intubation; avoid suctioning immediately after dosing.

N6. b — Respiratory and metabolic acidosis
Hypoxia and CO2 retention cause respiratory and metabolic (lactic) acidosis.